BASELINE → TREATMENT → FOLLOW-UP

Your treatment, tests and checks

Required regimen tests, symptom-triggered investigations and cancer-response tests serve different purposes. Ask who books each test and who explains the result.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks	Full blood count, kidney and liver tests; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing before the first fluorouracil treatment unless already tested. An electrocardiogram (ECG) is indicated if heart function is compromised. Hearing tests and creatinine clearance are arranged when clinically indicated; discuss any hearing loss, tinnitus or nerve symptoms.	Full blood count, kidney and liver tests before each cycle. Report vomiting, fluid intake, urine output, hearing change and nerve symptoms before treatment; further checks depend on results and symptoms.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Continue review of any kidney, hearing, nerve, mouth or nutritional problems. The team sets blood tests, examinations and imaging after treatment; no universal scan interval or blood tumour-marker panel is specified.
Treatment and appointments	Confirm the diagnosis, aim, prescription and actual calendar.	Cisplatin 25 mg/m² runs over 2 hours on each of Days 1–4, with prescribed hydration before each dose. Fluorouracil 1,000 mg/m² runs over 22 hours on each of Days 1–4. These are four daily cisplatin doses and four daily fluorouracil infusions; confirm the daily attendance or inpatient arrangement and pump-change times with your unit.	Follow the written home plan and keep pump or line appointments where applicable.	Both medicines are given on Days 1–4 of each 28-day cycle. Treatment continues while appropriate, until the cancer progresses or side effects become unacceptable; this protocol does not set a fixed number of cycles.	Record the last dose and follow-up appointments.
Mouth, weight and swallowing	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.
Cancer assessment	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00314 v4b: official regimen

Tests in your plan

Full blood count

WHEN

At baseline and before each cycle

Checks infection-fighting cells, oxygen-carrying cells and platelets.

Kidney and liver tests

WHEN

At baseline and before each cycle

Guide safe treatment and identify organ problems.

DPD assessment

WHEN

Before the first fluorouracil treatment unless already tested

Checks for reduced ability to break down fluorouracil; results affect treatment choice or dose. Testing cannot exclude every severe reaction. Kidney impairment can affect one blood-based test, so the team chooses the appropriate method.

Hearing assessment

If part of your plan

WHEN

At baseline when clinically indicated; review new hearing or balance symptoms during treatment

Investigates hearing loss or tinnitus and helps judge platinum-related risk.

Electrocardiogram (ECG)

If part of your plan

WHEN

Before treatment if heart function is compromised; additional assessment if symptoms warrant

Records the heart’s electrical activity. New chest symptoms require assessment even after a reassuring earlier test.

Imaging and tumour-specific tests

If part of your plan

WHEN

As individually arranged for the disease and symptoms; no universal marker panel or imaging interval

Assesses the cancer rather than whether a treatment-day blood count is safe.

THE WHOLE TREATMENT JOURNEY

Cisplatin + fluorouracil · 28-day cycle: treatment and checks

Locally advanced or metastatic head and neck cancer under NCCP 00314; confirm the individual treatment aim and relationship to any separately planned local treatment.

Both medicines are given on Days 1–4 of each 28-day cycle. Treatment continues while appropriate, until the cancer progresses or side effects become unacceptable; this protocol does not set a fixed number of cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests; DPD testing before the first fluorouracil treatment unless already tested. An ECG is indicated if heart function is compromised. Hearing tests and creatinine clearance are arranged when clinically indicated; discuss any hearing loss, tinnitus or nerve symptoms.	Full blood count, kidney and liver tests before each cycle. Report vomiting, fluid intake, urine output, hearing change and nerve symptoms before treatment; further checks depend on results and symptoms.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Continue review of any kidney, hearing, nerve, mouth or nutritional problems. The team sets blood tests, examinations and imaging after treatment; no universal scan interval or blood tumour-marker panel is specified.
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Mouth, weight and swallowing NCCP	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Cancer assessment NCCP	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

What each check is for

1 Combination treatment

Follow your agreed treatment calendar

Cisplatin and fluorouracil are used to treat head and neck cancer. The treatment aim depends on the extent of the cancer and your wider plan; ask your team to record whether the aim is disease control, symptom relief or part of another treatment strategy.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-10 · Source reconciled draft; independent clinical, oncology-pharmacy and patient/PPI review pending · SACTCheck v0.84.47